Medical Drug Clinical Criteria

Subject:	Monoclonal Antibodies to Interleukin-23		
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Table of Contents

Overview | Coding | References

Clinical criteria | Document history

Overview

This document addresses the use of monoclonal antibodies which bind to the interleukin-23 (IL-23) cytokine and disrupt its interaction with the IL-23 receptor thereby inhibiting the release of proinflammatory cytokines and chemokines. IL-23 inhibitors are approved for the treatment of plaque psoriasis. Agents addressed in this clinical criteria document include:

- Omvoh (mirikizumab-mrkz)
- Ilumya (tildrakizumab-asmn)
- Tremfya (guselkumab)
- Skyrizi (risankizumab-rzaa)

Plaque Psoriasis (otherwise known as psoriasis vulgaris): The American Academy of Dermatology (AAD) and National Psoriasis Foundation (NPF) published joint guidelines on the management and treatment of psoriasis with biologics. The guidelines do not include a treatment algorithm or compare biologics to each other or conventional therapy. The guideline notes that patients with mild-moderate disease may be adequately controlled with topical therapy and/or phototherapy while moderate to severe disease may necessitate treatment with a biologic. Moderate to severe disease is defined as involvement in greater than or equal to 3% of body surface area (BSA) or involvement in sensitive areas that significantly impact daily function (such as palms, soles of feet, head/neck, or genitalia). Tumor necrosis factor inhibitor (TNFi) biologics, ustekinumab, IL17 inhibitors, and IL23 inhibitors are all recommended as monotherapy treatment options for adult patients with moderate to severe plaque psoriasis.

Psoriatic Arthritis: The American College of Rheumatology (ACR) guidelines recommend that initial treatment of patients with active severe PsA or concomitant psoriasis should include a TNFi biologic over an oral small molecule (OSM; including methotrexate, sulfasalazine, cyclosporine, leflunomide, and apremilast). For initial therapy, OSMs are preferred over IL-17 and ustekinumab; and may be considered over TNFi biologics in mild to moderate disease without comorbid conditions or in those who prefer oral therapy. Recommendations involving biologics over OSMs as first line therapy are conditional and based on low quality evidence. Evidence cited includes indirect comparisons of placebo-controlled trials, studies with open-label design, and extrapolation from studies in plaque psoriasis. Furthermore, most pivotal trials for TNFi biologics included a study population that were DMARD experienced. Overall, there is a lack of definitive evidence for the safety and efficacy of biologic drugs over conventional therapy for the initial treatment of most patients with psoriatic arthritis. The ACR guidelines precede FDA approval of guselkumab and risankizumab for psoriatic arthritis.

Crohn Disease: Both the American College of Gastroenterology (ACG) and the American Gastroenterological Association (AGA) Guidelines recommend or suggest TNFi , vedolizumab, ustekinumab, risankizumab, guselkumab, mirikizumab, and upadacitinib for induction and maintenance of remission for patients with moderately to severely active UC. The AGA Guidelines provide recommendations for the positioning of advanced therapies. The AGA suggests using a higher efficacy medication (infliximab, adalimumab, vedolizumab, ustekinumab, risankizumab, mirikizumab, guselkumab) rather than a lower efficacy medication (certolizumab pegol, upadacitinib) in individuals who are naïve to advanced therapies. For individuals who have previously been exposed to one or more advanced therapies, particularly TNFi, AGA suggests using a higher efficacy medication (adalimumab, risankizumab, guselkumab, upadacitinib) or an intermediate efficacy medication (mirikizumab, ustekinumab) rather than a lower efficacy medication (certolizumab pegol, vedolizumab).).

Ulcerative Colitis: Both the American College of Gastroenterology (ACG) and the American Gastroenterological Association (AGA) Guidelines recommend or suggest TNFi, ozanimod, etrasimod, ustekinumab, guselkumab, mirikizumab, risankizumab, tofacitinib, upadacitinib, and vedolizumab for induction and maintenance of remission for patients with moderately to severely active UC. The AGA Guidelines provide recommendations for the positioning of advanced therapies. The AGA suggests using a higher efficacy medication (infliximab, vedolizumab, ozanimod, etrasimod, upadacitinib, risankizumab, guselkumab) or intermediate efficacy medication (golimumab, ustekinumab, tofacitinib, mirikizumab) medication rather than a lower efficacy medication (adalimumab) in individuals who are naïve to advanced therapies. For individuals who have previously been exposed to one or more advanced therapies, particularly TNFi, AGA suggests using a higher efficacy medication (tofacitinib, upadacitinib, ustekinumab) or an intermediate efficacy medication (mirikizumab, risankizumab, guselkumab) rather than a lower efficacy medication (adalimumab, vedolizumab, ozanimod, etrasimod).

Clinical Criteria

When a drug is being reviewed for coverage under a member's medical benefit plan or is otherwise subject to clinical review (including prior authorization), the following criteria will be used to determine whether the drug meets any applicable medical necessity requirements for the intended/prescribed purpose.

Ilumya (tildrakizumab-asmn)

Initial requests for Ilumya (tildrakizumab-asmn) may be approved for the following:

I. Plaque psoriasis (Ps) when each of the following criteria are met:
   A. Individual is 18 years of age or older with chronic moderate to severe (that is, extensive or disabling) plaque Ps with either of the following (AAD 2019):
      1. Plaque Ps involving three percent (3%) body surface area (BSA) or greater; **OR**
      2. Plaque Ps involving less than three percent (3%) BSA involving sensitive areas or areas that significantly impact daily function (such as palms, soles of feet, head/neck, or genitalia); **AND**
   B. Individual has had an inadequate response to, or is intolerant of phototherapy or other systemic therapy (such as acitretin, cyclosporine, or methotrexate); **OR**
   C. Individual has a contraindication to phototherapy, acitretin, cyclosporine, and methotrexate.

Continuation requests for Ilumya (tildrakizumab-asmn) may be approved if the following criteria are met:

I. Individual has been receiving and is maintained on a stable dose of Ilumya; **AND**
II. There is clinically significant improvement or stabilization in clinical signs and symptoms of disease.

Requests for Ilumya (tildrakizumab-asmn) may not be approved for the following:

I. In combination with topical or oral JAK inhibitors, apremilast, deucravacitinib, ozanimod, etrasimod, or any of the following immunomodulators: TNF antagonists, other IL-23 inhibitors, IL-17 inhibitors, vedolizumab, ustekinumab, abatacept, IL-1 inhibitors, IL-6 inhibitors, rituximab or natalizumab; **OR**
II. When the above criteria are not met and for all other indications.

Omvoh (mirikizumab-mrkz)

Initial requests for Omvoh (mirikizumab-mrkz) may be approved for the following:

I. Ulcerative colitis (UC) when the following criterion is met:
   A. Individual is 18 years of age or older with moderate to severe UC;

OR

II. Crohn Disease (CD) when each of the following criterion is met:
   A. Individual is 18 years of age or older with moderate to severe CD.

Continuation requests for Omvoh (mirikizumab-mrkz) [intravenous and subcutaneous] may be approved if the following criteria are met:

I. Individual has been receiving and is maintained on a stable dose of Omvoh; **AND**
II. There is clinically significant improvement or stabilization in clinical signs and symptoms of disease.

Requests for Omvoh (mirikizumab-mrkz) may not be approved for the following:

I. In combination with topical or oral JAK inhibitors, apremilast, deucravacitinib, ozanimod, etrasimod, or any of the following immunomodulators: TNF antagonists, other IL-23 inhibitors, IL-17 inhibitors, vedolizumab, ustekinumab, abatacept, IL-1 inhibitors, IL-6 inhibitors, rituximab or natalizumab; **OR**
II. When the above criteria are not met and for all other indications.

Skyrizi (risankizumab-rzaa)

Initial requests for Skyrizi (risankizumab-rzaa) may be approved for the following:

I. Plaque psoriasis (Ps) when each of the following criteria are met:
   A. Individual is 6 years of age or older with chronic moderate to severe (that is, extensive or disabling) plaque Ps with either of the following (AAD 2019):
      1. Plaque Ps involving three percent (3%) body surface area (BSA) or greater; **OR**
      2. Plaque Ps involving less than three percent (3%) BSA involving sensitive areas or areas that significantly impact daily function (such as palms, soles of feet, head/neck, or genitalia); **AND**
   B. Individual has had an inadequate response to, or is intolerant of phototherapy or other systemic therapy (such as acitretin, cyclosporine, or methotrexate); **OR**
   C. Individual has a contraindication to phototherapy, acitretin, cyclosporine, and methotrexate;

OR

II. Psoriatic arthritis (PsA) when each of the following criteria are met:
   A. Individual is 6 years of age or older with moderate to severe PsA; **AND**
   B. Individual has had an inadequate response to, or is intolerant of conventional therapy [nonbiologic DMARDs (such as methotrexate, sulfasalazine, cyclosporine or leflunomide)]; **OR**
   C. Individual has a contraindication to methotrexate, sulfasalazine, cyclosporine, and leflunomide;

OR

III. Crohn Disease (CD) when each of the following criterion is met:
   A. Individual is 18 years of age or older with moderate to severe CD;

OR

IV. Ulcerative colitis (UC) when the following criterion is met:
   A. Individual is 18 years of age or older with moderate to severe UC**.**

Continuation requests for Skyrizi (risankizumab-rzaa) may be approved if the following criteria are met:

I. Individual has been receiving and is maintained on a stable dose of Skyrizi; **AND**
II. There is clinically significant improvement or stabilization in clinical signs and symptoms of disease.

Requests for Skyrizi (risankizumab-rzaa) may not be approved for the following:

I. In combination with topical or oral JAK inhibitors, apremilast, deucravacitinib, ozanimod, etrasimod, or any of the following immunomodulators: TNF antagonists, other IL-23 inhibitors, IL-17 inhibitors, vedolizumab, ustekinumab, abatacept, IL-1 inhibitors, IL-6 inhibitors, rituximab or natalizumab; **OR**
II. When the above criteria are not met and for all other indications.

Tremfya (guselkumab)

Initial requests for Tremfya (guselkumab) may be approved for the following:

I. Plaque psoriasis (Ps) when each of the following criteria are met:
   A. Individual is 6 years of age or older weighing at least 40 kg with chronic moderate to severe (that is, extensive or disabling) plaque Ps with either of the following (AAD 2019):
      1. Plaque Ps involving three percent (3%) body surface area (BSA) or greater; **OR**
      2. Plaque Ps involving less than three percent (3%) BSA involving sensitive areas or areas that significantly impact daily function (such as palms, soles of feet, head/neck, or genitalia); **AND**
   B. Individual has had an inadequate response to, or is intolerant of phototherapy or other systemic therapy (such as acitretin, cyclosporine, or methotrexate); **OR**
   C. Individual has a contraindication to phototherapy, acitretin, cyclosporine, and methotrexate;

OR

II. Psoriatic arthritis (PsA) when each of the following criteria are met:
   A. Individual is 6 years of age or older weighing at least 40 kg with moderate to severe PsA; **AND**
   B. Individual has had an inadequate response to, or is intolerant of conventional therapy [nonbiologic DMARDs (such as methotrexate, sulfasalazine, cyclosporine or leflunomide)]; **OR**
   C. Individual has a contraindication to methotrexate, sulfasalazine, cyclosporine, and leflunomide;

OR

III. Crohn Disease (CD) when each of the following criteria are met:
   A. Individual is 18 years of age or older with moderate to severe CD;

OR

IV. Ulcerative colitis (UC) when the following criterion is met:
   A. Individual is 18 years of age or older with moderate to severe UC.

Continuation requests for Tremfya (guselkumab) may be approved if the following criteria are met:

I. Individual has been receiving and is maintained on a stable dose of Tremfya; **AND**
II. There is confirmation of clinically significant improvement or stabilization in clinical signs and symptoms of disease.

Requests for Tremfya (guselkumab) may not be approved for the following:

I. In combination with topical or oral JAK inhibitors, apremilast, deucravacitinib, ozanimod, etrasimod, or any of the following immunomodulators: TNF antagonists, other IL-23 inhibitors, IL-17 inhibitors, vedolizumab, ustekinumab, abatacept, IL-1 inhibitors, IL-6 inhibitors, rituximab or natalizumab; **OR**
II. When the above criteria are not met and for all other indications.

Step Therapy

Note: When a monoclonal antibody to interleukin-23 is deemed approvable based on the clinical criteria above, the benefit plan may have additional criteria requiring the use of a preferred[1] agent or agents.

Omvoh Step Therapy

A list of preferred targeted immune modulator(s) is available here.

Commercial requests for diagnosis of Crohn Disease and Ulcerative Colitis:

Requests for Omvoh may be approved if the following criteria are met:

I. Individual is currently on Omvoh;
OR
II. Individual has had a trial and inadequate response or intolerance to TWO preferred agent(s) (A trial of multiple products with the same active ingredient counts as a trial of ONE preferred agent);
OR
III. Information is provided for why the individual is unable to use ALL preferred agents (or all remaining preferred agents if individual has tried any preferred agent) due to one of the following:
   A. The individual is subject to a warning or contraindication that appears in the labeling of ALL preferred products and is not included in the labeling of the non-preferred product; **OR**
   B. None of the preferred products have activity against the individual's concomitant clinical condition which is covered by the non-preferred product; **OR**
   C. Remaining preferred agents are TNF antagonists and individual has already been exposed to 1 or more TNF antagonists (i.e. individual is unable to use all preferred agents other than TNF antagonists due to one of the reasons above).

[1]Preferred, as used herein, refers to agents that were deemed to be clinically comparable to other agents in the same class or disease category but are preferred based upon clinical evidence and cost effectiveness.

Quantity Limits

Ilumya (tildrakizumab-asmn) Quantity Limit

Drug	Limit
Ilumya (tildrakizumab-asmn) 100 mg/mL	1 prefilled syringe per 84 days (12 weeks)
Override Criteria	
*Initiation of therapy for Plaque Psoriasis (Ps): May approve up to 1 additional syringe (100 mg/mL) in the first 28 days (4 weeks) of treatment.	

Omvoh (mirikizumab-mrkz)

Drug	Limit
Omvoh (mirikizumab-mrkz) 100 mg/mL prefilled pen/syringe	2 pens/syringes [1 carton] per 28 days (4 weeks)
Omvoh (mirikizumab-mrkz) 200 mg/mL prefilled pen/syringe	1 pens/syringe [1 carton] per 28 days (4 weeks)

Omvoh (mirikizumab-mrkz) 200 mg/2 mL + 100 mg/mL prefilled pen/syringe	2 pens/syringes [1 carton] per 28 days (4 weeks)
Omvoh (mirikizumab-mrkz) 300 mg/15 mL single-dose vial	9 vials total; one time fill

Skyrizi (risankizumab-rzaa) Quantity Limit

Drug	Limit
Skyrizi (risankizumab-rzaa) 55 mg/0.37mL syringe*	1 prefilled syringe [1 carton] per 84 days (12 weeks)
Skyrizi (risankizumab-rzaa) 90 mg/mL syringe^	2 prefilled syringes per 56 days (8 weeks)
Skyrizi (risankizumab-rzaa) 180 mg/ 1.2 mL syringe^	1 prefilled syringe per 56 days (8 weeks)
Skyrizi (risankizumab-rzaa) 150 mg/mL syringe/pen*	1 prefilled syringe/pen [1 carton] per 84 days (12 weeks)
Skyrizi (Risankizumab-rzaa) 180 mg/ 1.2 mL prefilled cartridge with on-body injector	1 kit per 56 days (8 weeks)
Skyrizi (Risankizumab-rzaa) 360 mg/ 2.4 mL prefilled cartridge with on-body injector	1 kit per 56 days (8 weeks)
Skyrizi (Risankizumab-rzaa) 600 mg/ 10 mL single-dose vial	6 vials total; one time fill
Override Criteria	
*Initiation of therapy for Plaque Psoriasis (Ps) or Psoriatic Arthritis (PsA): May approve 1 additional carton [one 150 mg pen/syringe OR one 55 mg syringe] in the first 28 days (4 weeks) of treatment. ^Requests for up to 4 of the 90 mg/mL prefilled syringes OR up to two of the 180 mg/mL prefilled syringes per 56 days (8 weeks) may be approved if the following criteria are met: I. Information has been provided for the clinical necessity of the prefilled syringe and why the individual is unable to use the 360 mg prefilled cartridge with on-body injector.	

Tremfya (guselkumab) Quantity Limit

Drug	Limit
Tremfya (guselkumab) 100 mg/mL pen/syringe*	1 prefilled pen/syringe per 56 days (8 weeks)
Tremfya (guselkumab) 200 mg/2 mL pen/syringe^	1 pen/syringe per 28 days (4 weeks)
Tremfya (guselkumab) Induction Pack for Ulcerative Colitis or Crohn Disease 200 mg/2 mL pen^	3 packs total; one time supply
Tremfya (guselkumab) 200 mg/20 mL single-dose vial	3 vials total; one time fill
Override Criteria	
*Initiation of therapy for Plaque Psoriasis (Ps) or Psoriatic Arthritis (PsA): May approve up to 1 additional pen/syringe (100 mg/mL) in the first 28 days (4 weeks) of treatment. ^Initiation of therapy for Ulcerative colitis or Crohn Disease: May approve 3 Induction Packs OR up to 6 total (3 additional) pens/syringes (200mg/2mL) in the first 8 weeks of treatment; to be administered as one induction pack OR 2 (two) pens/syringes (200mg/2mL) at week 0, week 4, and week 8.	

Coding

The following codes for treatments and procedures applicable to this document are included below for informational purposes. Inclusion or exclusion of a procedure, diagnosis or device code(s) does not constitute or imply member coverage or provider reimbursement policy. Please refer to the member's contract benefits in effect at the time of service to determine coverage or non-coverage of these services as it applies to an individual member.

Skyrizi (risankizumab-rzaa) and Tremfya (guselkumab)

HCPCS

C9399	Unclassified drugs or biologicals [when specified as Skyrizi (risankizumab-rzaa) subcutaneous]
J1628	Injection, guselkumab, 1 mg [Tremfya]
J2327	Injection, risankizumab-rzaa, intravenous, 1 mg [Skyrizi]
J3590	Unclassified biologics [when specified as Skyrizi (risankizumab-rzaa) subcutaneous]

ICD-10 Diagnosis

K50.00-K50.919	Crohn disease
K51.00-K51.919	Ulcerative Colitis
L40.9-L40.9	Psoriasis

Ilumya (tildrakizumab-asmn)

HCPCS

J3245	Injection, tildrakizumab, 1 mg [Ilumya]

ICD-10 Diagnosis

L40.0	Psoriasis vulgaris
L40.1	Generalized pustular psoriasis
L40.2	Acrodermatitis continua
L40.3	Pustulosis palmaris et plantaris
L40.4	Guttate psoriasis
L40.8	Other psoriasis
L40.9	Psoriasis, unspecified

Omvoh (mirikizumab-mrkz)

HCPCS

J2267	Injection, mirikizumab-mrkz, 1 mg [Omvoh]

ICD-10 Diagnosis

K50.00-K50.919	Crohn disease
K51.00-K51.919	Ulcerative colitis

Document History

Revised: 08/14/2026
Document History:

- 08/14/2026 – Annual Review: Update Skyrizi clinical criteria for expanded age indications and add quantity limit for new strength; clarify may not approve criteria. Coding Reviewed: No changes.
- 03/26/2026 – Step therapy table updates.
- 02/20/2026 – Select Review: Update may not approve criteria; remove infection, tuberculosis testing, and phototherapy combination exclusions as applicable. Coding Reviewed: No changes.
- 11/14/2025 – Annual Review: Remove prerequisite conventional therapy requirements for CD and UC; update Tremfya clinical criteria with new pediatric indications; clarify phototherapy exclusion applies to psoriasis and psoriatic arthritis; clarify BSA requirements for psoriasis; add quantity limit for new Skyrizi and Omvoh dosage forms; update Tremfya quantity limit for induction pack; clarify quantity limits as one time fill as applicable. Step therapy update. Coding Reviewed: Added HCPCS NOC C9399 and J3590 for Skyrizi subcutaneous.
- 10/01/2025 – Step therapy table updates.
- 05/16/2025 – Select Review: Update Tremfya clinical criteria with new indications for Crohn's disease; add quantity limit for new Tremfya induction pack and add override for induction dosing in Crohn's disease. Step therapy table updates. Coding Reviewed: Separated HCPCS and ICD-10-CM according to drug. Added ICD-10-CM K50.00-K50.919 for Tremfya. Consolidated L40.0-L40.9 into one code range and updated description for Skyrizi, Tremfya.
- 05/01/2025 – Step therapy table updates.
- 2/21/2025 – Select Review: Update Omvoh clinical criteria with new indication for Crohn's disease; add quantity limit for new strength; increase quantity limit for vials. Coding Reviewed: Consolidated individual code

ranges for Crohn's disease into a single range K50.00-K50.919 and updated description. Added Omvoh to K50.00-K50.919. Consolidated individual codes for arthropathic psoriasis into one range L40.50-L40.59. Removed Ilumya from L40.50-L40.59.

- 11/15/2024 – Annual Review: Update and clarify quantity limits based on approved dosage forms. 09/09/2024 – Select Review: Update Tremfya clinical criteria with new indication for ulcerative colitis; add quantity limit for new Tremfya dosages. Coding Reviewed: Designate Skyrizi for Chron's, UC, and Psoriasis indications, Tremfya for UC and Psoriasis indications, Omvoh for UC indications, and Ilumya for Psoriasis indications in parentheses next to diagnosis descriptions. Step therapy table updates. Coding reviewed: No changes.
- 08/16/2024 – Select Review: Update Skyrizi clinical criteria to include new indication for Ulcerative Colitis; update Skyrizi quantity limit to remove obsolete strength, include new 90 mg strength with override, and increase vial quantity limit for UC induction dosing. Step therapy table updates. Coding Reviewed: Ulcerative Colitis indication already in document. Add ICD-10-CM L40.1, L40.2, L40.3, L40.4, L40.54 and changed naming of L40.0 from Plaque psoriasis to Psoriasis vulgaris.
- 07/01/2024 – Add new step therapy table.
- 11/17/2023 – Annual Review: Add clinical criteria and quantity limits to new agent Omvoh; add etrasimod to combination use exclusion for consistency; update contraindication to prior therapy language for clarity; update Crohn's disease criteria to separate criteria for IV induction dose or subcutaneous therapy; clarify repeat TB testing requirements; add continuation of use language; wording and formatting updates. Coding Reviewed: Added J3590 for Omvoh. Added ICD-10-DM K51.00-K51.919 (Omvoh). Removed HCPCS J3490, J3590, C9399. Effective 4/1/24 Added HCPCS C9168 for Omvoh. Effective 7/1/2024 Added HCPCS J2267 for Omvoh. Removed HCPCS J3590, C9168.
- 11/18/2022 – Annual Review: Update combination exclusion list to include additional agents and specify biologic immunomodulators; include additional conventional therapy examples per guidelines; add quantity limit for new Skyrizi dosage form; wording and formatting updates. Coding Reviewed: Added HCPCS C9399. Effective 1/1/2023 Added HCPCS J2327 for Skyrizi. Added ICD-10-CM K50.00-K50.019, K50.10-K50.119, K50.80-K50.819, K50.90-K50.919.
- 08/19/2022 – Select Review: Add new Crohn's disease indication to Skyrizi clinical criteria; add quantity limit for new dosage forms of Skyrizi. Coding Reviewed: Added wording change to L40.0. Removed ICD-10-CM L40.1, L40.2, L40.3, L40.4, L40.50-L40.59. Added ICD-10-CM L40.50, L40.51, L40.52, L40.53, L40.59.
- 08/19/2022 – Select Review: Add new Crohn's disease indication to Skyrizi clinical criteria; add quantity limit for new dosage forms of Skyrizi.
- 02/25/2022 – Select Review: Add new psoriatic arthritis indication to Skyrizi clinical criteria and quantity limit; wording and formatting changes. Coding Reviewed: No changes.
- 11/19/2021 – Annual Review: Clarify tuberculosis testing language; remove option of trial of TNF for consistency for Tremfya; update exclusion list for combination use. Coding reviewed: No changes.
- 06/14/2021 – Select Review: Add quantity limit for new dosage form of Skyrizi. Coding Reviewed: No changes.
- 11/20/2020 – Annual Review: Add continuation of use section; add additional examples of combination use for clarity; update tuberculosis testing language. Coding Reviewed: No changes.
- 08/21/2020 – Select Review: Add new psoriatic arthritis indication to Tremfya clinical criteria; update quantity limit override. Coding Reviewed: Added ICD-10CM-L40.50-L40.59
- 11/15/2019 – Annual Review: Update definition of moderate psoriasis using BSA based on guidelines; update combination therapy criteria for consistency with other agents; wording and formatting changes. Coding reviewed: No Changes.
- 09/23/2019 - Administrative update to add drug specific quantity limit.
- 05/17/2019 – Select Review: Add new clinical criteria and quantity limit for new agent Skyrizi. Update Tremfya quantity limit to include new autoinjector formulation per label. Coding Review: Added J3490 and J3590 for Skyrizi.
- 11/16/2018 – Annual Review: Initial P&T review of Monoclonal Antibodies to Interleukin-23 Clinical Guideline. Update clinical criteria to delete requirement agent is being used "to reduce signs and symptoms, maintain clinical response", etc. Wording and formatting changes to criteria for consistency. HCPCS and ICD-10 Coding Review: Deleted C9029, J3490, J3590. Added J1628, J3245.

References

1. DailyMed. Package inserts. U.S. National Library of Medicine, National Institutes of Health website. http://dailymed.nlm.nih.gov/dailymed/about.cfm. Accessed: June 26, 2026.
2. Merative™ Micromedex® DRUGDEX® (electronic version). Merative, Ann Arbor, Michigan, USA
3. Scott FI, Ananthakrishnan AN, Click B, Agrawal M, Syal G, Haydek JP, Yuan Y, Kappelman MD, Lewis JD, Singh S. AGA Living Clinical Practice Guideline on the Pharmacologic Management of Moderate-to-Severe Crohn's Disease. Gastroenterology. 2025;169(7):1397-1448.

4. Singh S, Loftus EV, Limketkai BN et al. AGA Living Clinical Practice Guideline on Pharmacological Management of Moderate-to-Sever Ulcerative Colitis. Gastroenterology 2024; 167:130-1343.
5. Lexi-Comp ONLINE™ with AHFS™, Hudson, Ohio: Lexi-Comp, Inc.; 2025; Updated periodically.
6. Lichtenstein GR, Loftus EV, Afzali, A et al. ACG Clinical Guideline: Management of Crohn's Disease in Adults. The American Journal of Gastroenterology 120(6):p 1225-1264, June 2025.
7. Menter A, Strober BE, Kaplan DH, et al. Joint AAD-NPF guidelines of care for the management and treatment of psoriasis with biologics. *J Am Acad Dermatol*. 2019; 80: 1029-72.
8. Rubin DT, Ananthakrishnan AN, Siegel CA et al. ACG Clinical Guideline Update: Ulcerative Colitis in Adults. The American Journal of Gastroenterology 120(6):p 1187-1224, June 2025.
9. Singh JA, Guyatt G, Ogdie A, et al. 2018 American College of Rheumatology/National Psoriasis Foundation Guideline for the Treatment of Psoriatic Arthritis. Arthritis Rheum. 2019; 71(1): 5-32.

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CC-0050 Monoclonal Antibodies to Interleukin-23

Commercial Medical Benefit

Ulcerative Colitis		
Effective Date	**Preferred Agents**	**Non-Preferred Agents**
10/1/2025	Avsola Remicade Unbranded Infliximab Entyvio Selarsdi IV Stelara IV Unbranded Ustekinumab IV Skyrizi IV Tremfya IV	Omvoh

Crohn Disease		
Effective Date	**Preferred Agents**	**Non-Preferred Agents**
10/1/2025	Avsola Remicade Unbranded Infliximab (Remicade) Entyvio Selarsdi IV Stelara IV Unbranded Ustekinumab IV (Stelara) Skyrizi IV Tremfya IV	Omvoh